PROPOSAL FOR DISCUSSION

Histology Redesigned

Closing handoff gaps in the histology workflow, measured first

Prepared for: **Director of Operations, Platinum Dermatology**

Jorge Rodriguez, histology assistant · [meeting date]



Synthetic demonstration · fictional data · not endorsed by anyone

What I see at the bench

1. The same details copied by hand at several handoffs
2. Office-to-doctor routing kept in people’s memory
3. Parts A and B sometimes arriving as separate orders
4. Clarification replies can wait; we don’t know why yet
5. Gross entry batched late; labels and “the bible” by hand

My observations as an employee, not measured rates.

Observations, not measurements

I don’t know how often each one happens, or whether some already have a fix.

Step one is to measure.

Configure first, buy second, build last

1

Configure

Check what EMA and our existing tools already do

2

Buy

Look at existing products. Several LIS vendors advertise ModMed integrations: vendor claims, to check

3

Build

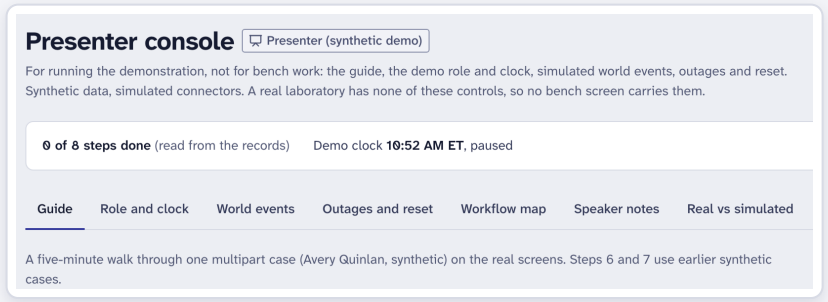
Only for gaps that remain, and only after measuring

A good outcome is the problem solved, even if this software isn't what solves it.

Vendor claim example: ligolab.com (LigoLab on its ModMed integration). A vendor claim, not independent evidence; to confirm with the vendor.

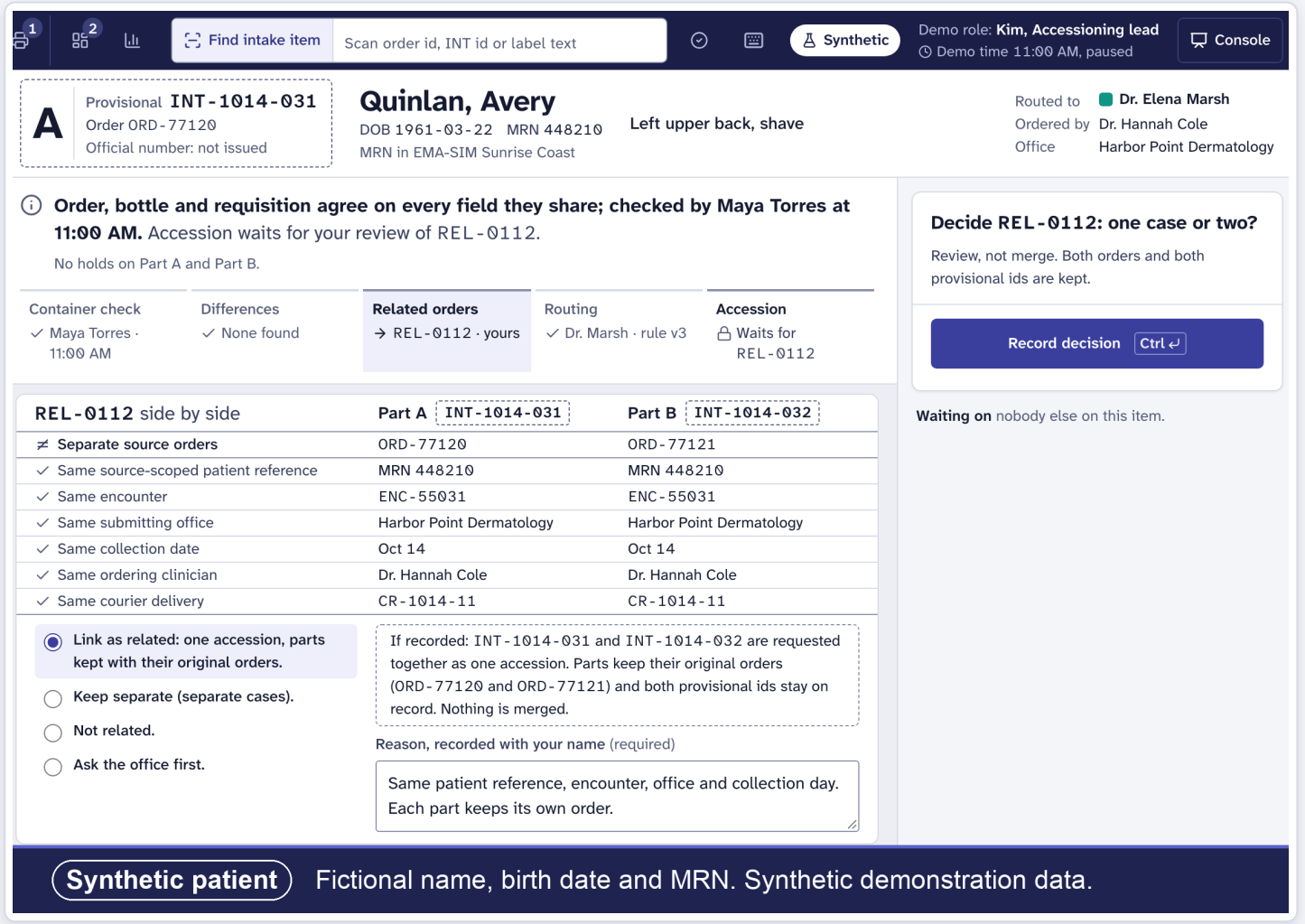
Demo, part 1: arrival and two orders for one patient

Guided demo · steps 1–2 of 8



Presenter console: the eight-step guide

- Courier delivery checked against what was expected
- Routing by a written, versioned rule; an unknown office is flagged, not guessed
- Two orders, one patient: the system suggests, a person decides
- Nothing is merged; both original orders are kept

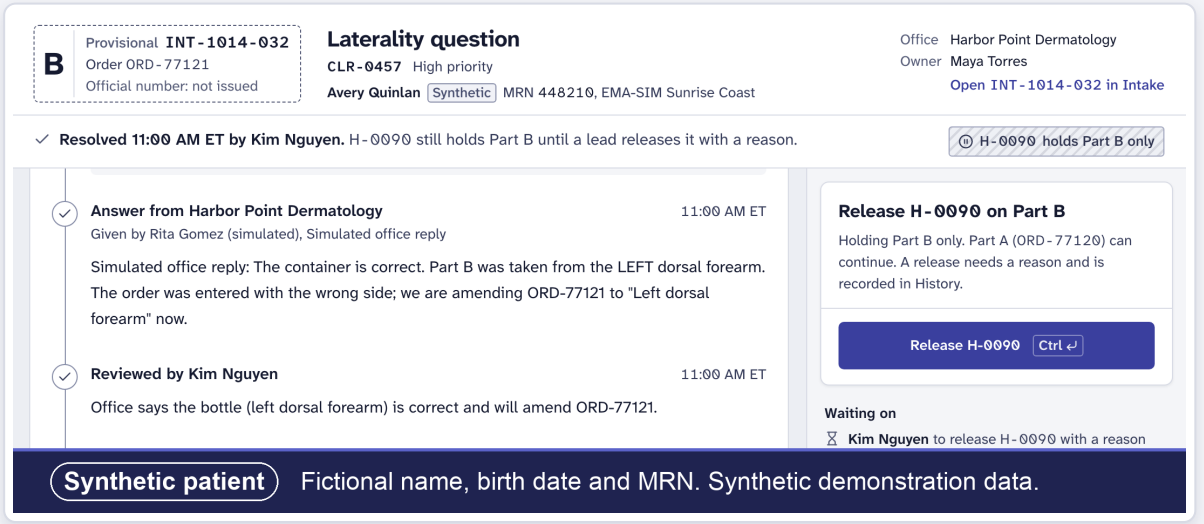


Intake: REL-0112, two source orders side by side; "Link as related" chosen, nothing merged

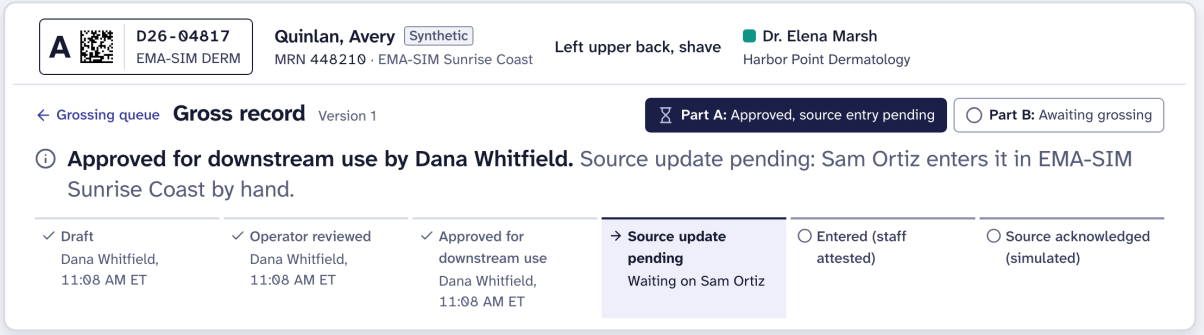
Demo, part 2: hold only what’s wrong

Guided demo · steps 3–5 of 8

- Bottle says left, order says right: one clarification, and only Part B is held
- A person reviews the office’s answer before the question is closed
- The hold comes off separately, with a reason, after the corrected order arrives
- Part A’s gross record moves on; Part B stays visibly unfinished



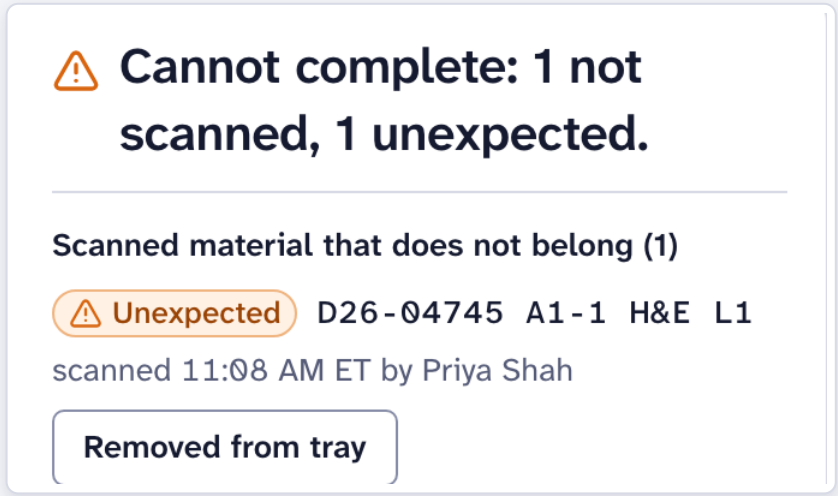
Clarification CLR-0457: answer reviewed; hold H-0090 still holds Part B only



Gross record, Part A approved for downstream use; Part B still awaiting grossing

Demo, part 3: physical checks, and what is still blocked

Guided demo · steps 6–8 of 8



A tray missing a slide can’t be marked complete

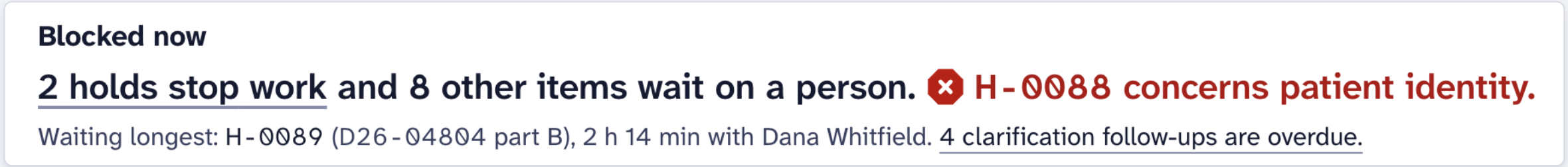


A wrong block scan is rejected, and nothing changes

Printer timeout

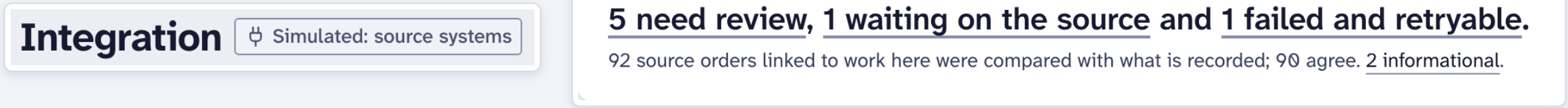
The screen shows “unknown”, never “printed”, until someone checks the printer. No reprint before that.

Not pictured



Operations: what is blocked, who owns the next step, how long it has waited. Made-up numbers, not lab results.

What’s real and what’s simulated



The Integration screen (two crops): simulated EMA messages, and what a person must still decide. The real EMA interface is still to confirm with ModMed.

Works today — on synthetic data: workflow rules, role checks, audit trail, backup and restore, health checks

Simulated — EMA, offices, couriers, printers, scanners and the drafting “assistant”

Blocked — real sign-in and hosting, until the organization provides them

Needs people to check — screen readers, paper printing, real hardware, touchscreens

About 800 automated checks pass, on synthetic data. **Not clinical validation.**

Synthetic demonstration data Fictional lab, staff, offices and patients · simulated connectors

Implementation: six phases, with gates where the plan can stop

0

Discovery and governance

Weeks 1–4

Decide: configure, buy, build or stop

◆

1

Synthetic sandbox at the lab

Weeks 5–10

Bench usability with real scanners and printers; no patient data

◆

2

Technical foundation

Weeks 8–22

Hosting, sign-in, security review, vendor interface (test only)

◆

3

Validation and training

Weeks 20–28

Under the lab's quality system, approved by the Medical Director

◆

4

Limited pilot

Weeks 28–36

One workflow, one bench; shadow first; weekly continue-or-stop reviews

5

Decision

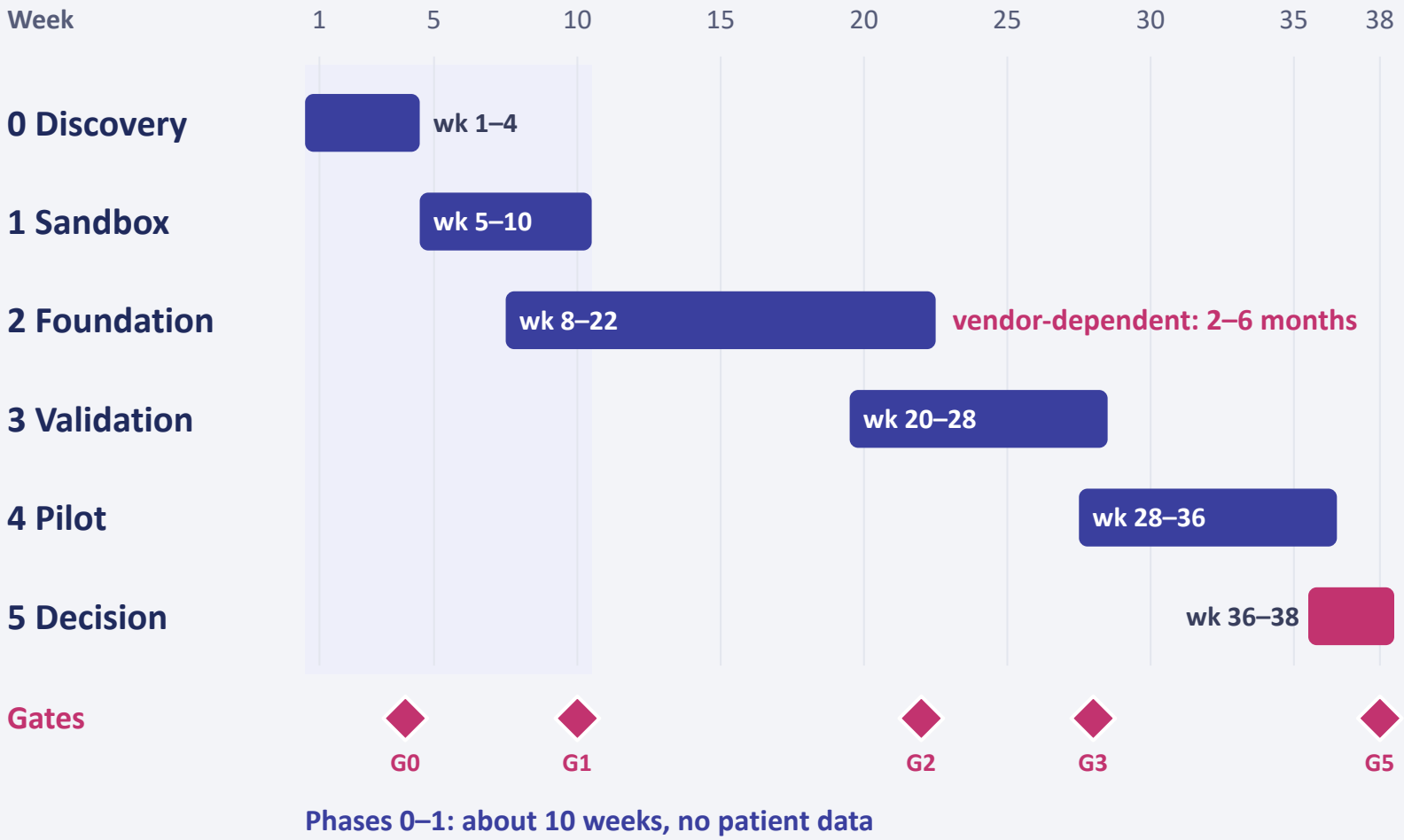
Weeks 36–38

Continue, expand, change or stop

Phases 0–3 end at a gate (◆) where the plan can stop; the pilot is reviewed every week.

Stopping, or configuring existing tools, is a success. Weeks from approval are rough estimates to refine in Phase 0. Phases overlap; vendor scoping runs without a contract, and nothing is signed or purchased before Gate 1.

Timeline (rough estimates, weeks from approval)



Phases 0–1: about 10 weeks, no patient data

Best case: about

7–9 months

to the end of a limited pilot, if a vendor test environment is ready by about week 15

Up to about 12 months with vendor delays

Cheapest decision-grade path

About 10 weeks

Phases 0–1 only, no patient data

This is the build path. If Phase 0 chooses configure or buy, the plan is redrawn with ModMed or the vendor.

What it takes (roles and time, rough estimates)

Across the whole build path. Phase 0 alone is on slide 12 (the ask). Ranges, not commitments.

Sponsor (Director of Operations)

about **1 h a week**

plus gate decisions

Medical Director

about **4–8 h**

plus gate reviews and validation approval

IT

about **40–100 h**

hosting, sign-in, backups, monitoring

Compliance/Privacy

about **10–30 h**

data handling review and agreements

Bench staff

sessions of **1–2 h**

usability, for 5–8 people; 1–2 h training per role

Validation

about **60–150 h**

testing and documentation, depending on the workflow

Unknown until quotes or estimates: vendor interface fees, connector development, external security test, hosting, a support contract or IT time after go-live, hardware and label stock.

Risks and safeguards

Risk	Safeguard
One developer (me)	A named developer before any interface work; a named owner before any pilot (internal IT, a contracted developer, or a vendor product instead)
A second system, double entry	EMA stays the system of record; the extra work is measured, not hidden
Interface delays or fees	Phases 0–1 need no interface; we can stop or switch at any gate
Wrong or stale information	Shows “unknown” and data age; fails closed; tamper-evident audit
Unproven safety	An independent security test; validation approved by the Medical Director

Audit check: the activity record has a check that would detect an edit. In the demo it uses a public key, so it only shows how the check works.

The ask: Phase 0 only

Four weeks of discovery and observation



At week 4: a written recommendation to

configure | **buy** | **build** | **stop**

No patient data

No software installed on lab systems

No purchase

Phase 0 effort (rough estimate)

- **You:** about 4 h over the 4 weeks, plus Gate 0
- **Medical Director, IT lead, Compliance/Privacy:** [to estimate at kickoff]
- **Me:** [to confirm with my manager, and who covers my bench]

What you would allow

- Observation, with staff consent
- A 30-minute small-group demo review
- Questions to ModMed or an LIS vendor, sent through [to confirm: the EMA administrator or IT lead]
- **No contract, no data. Not a request for money or system access.**

Where this could go

Vision, not a request

Today's ask is Phase 0 only.

A full lab system built on the same bench rules, working with EMA, not replacing it

A — Pilot
The current proposal, starting with Phase 0

B — Sign-out and result delivery
Pathologist workspace; results back to EMA

C — Core lab workflow
Billing support, special stains and IHC

D — Platform and migration
Production hosting; alongside B and C

Possible later steps. Each is a separate decision, made only on the evidence of the one before.

about 18–24 months Rough estimate

after the pilot, including validation and go-live:
the lab's own system of record for lab work
(EMA stays the practices' record)
with a funded team of about 5–8 people

about 3–5 years Rough estimate

after the pilot:
a product used by many labs

The pilot comes first

- Does it help the bench?
- Do staff want to use it?
- Can it connect to EMA?
- What does validation really take?

Rough planning ranges: rough estimates only, not quotes or commitments. Who would fund, own and support it is an open question.

Questions

Leave-behind: the two-page brief, the timeline, the FAQ and the one-page decision checklist

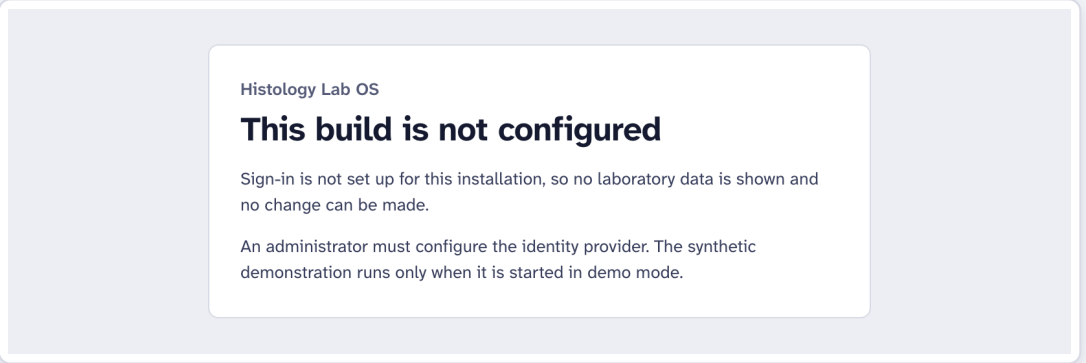
Jorge Rodriguez, histology assistant · [to confirm: preferred work contact]



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Appendix: evidence and sources

- **Latest verification (2026-10-24):** type check clean; 643/643 unit, integration and contract; 139/139 demo browser; 26/26 secured. Re-run on the frozen code before the meeting
- **Fails closed:** a build that isn't configured shows no data and opens no database
- **Public sources** on ModMed interfaces and CAP information-system requirements: to confirm with the vendor
- **Full documents:** pilot proposal, discovery checklist, production readiness



The locked build: the same software (Histology Lab OS) started without demo mode and without sign-in configured. Every page leads here: no data, no database opened, no changes possible.

Sources: public pages. Confirm each with the vendor, and check CAP items against the current checklist edition.

ModMed, HL7 connectivity: modmed.com/synapsys/hl7

ModMed, lab connection (partner) process: modmed.com/lab-connect

LigoLab on its ModMed integration (vendor claim): ligolab.com/post/…

ModMed, lab orders and results interface: modmed.com/?p=31624

ModMed API portal (FHIR R4, EMA API): portal.api.modmed.com

CAP information-system requirements, via MLO (2017): mlo-online.com/…/13009283/